Workflow choice: experiment and limits

Terminology

Reference outcomes are the acceptable complete or partial workflows under the frozen modeling specification. The independent reference evaluator checks them by exhaustive subset enumeration. It is separately implemented from the methods under test, but authored by the same developer; no external expert endorsement is implied.

Design

4 development and 14 internal confirmation families; 72 method executions. Whole matching dependency skeletons were grouped before comparative evaluation. Policy and action primitives remain shared.

Question

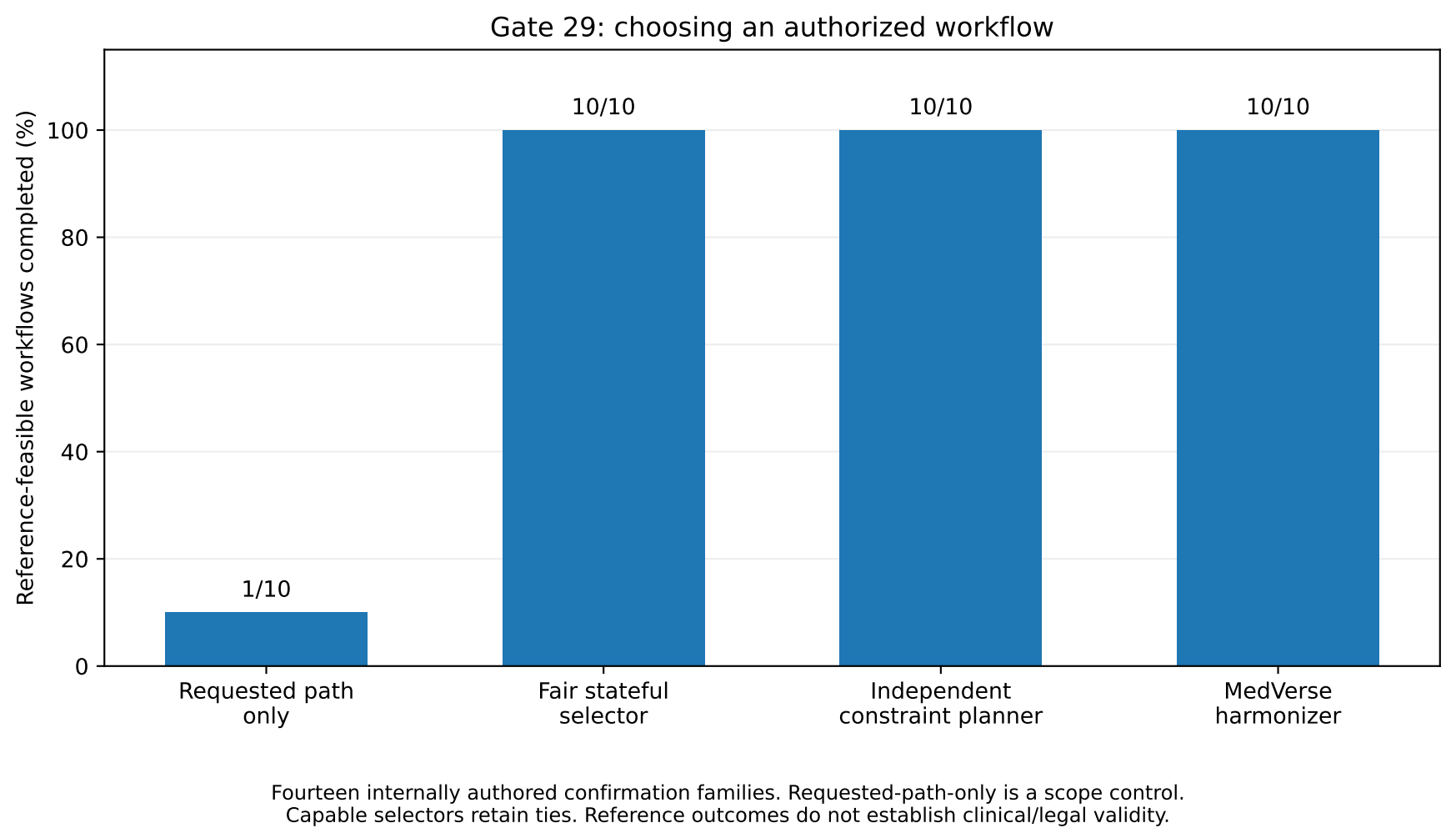
Can institution-owned alternative actions satisfy clinical, research, authority and DT-context obligations when the originally requested path cannot? The requested-path-only control is not a strong general planner. Fair stateful search and a separate constraint planner have the same authorized inputs and objective.

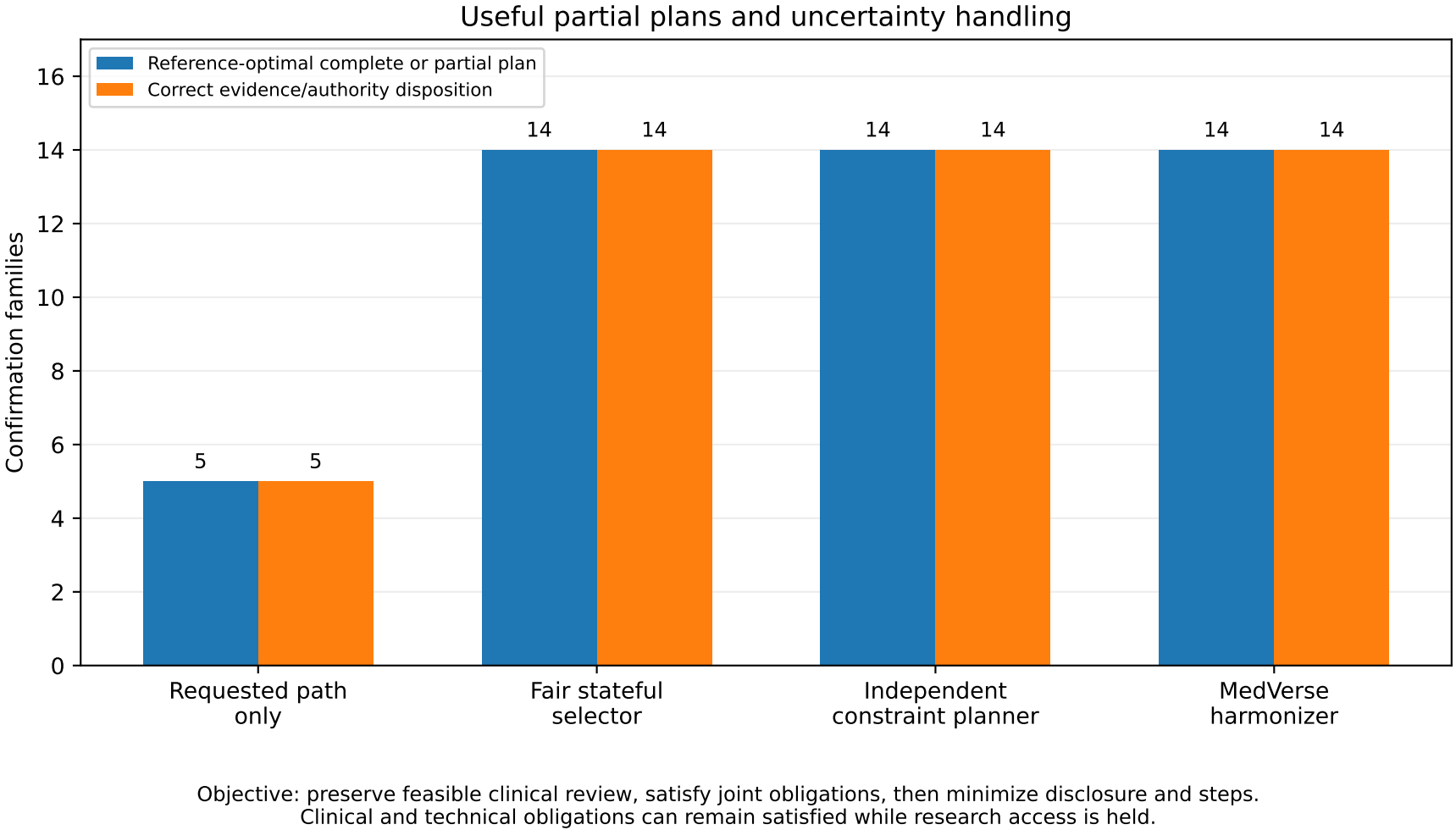
Engineering and evidence

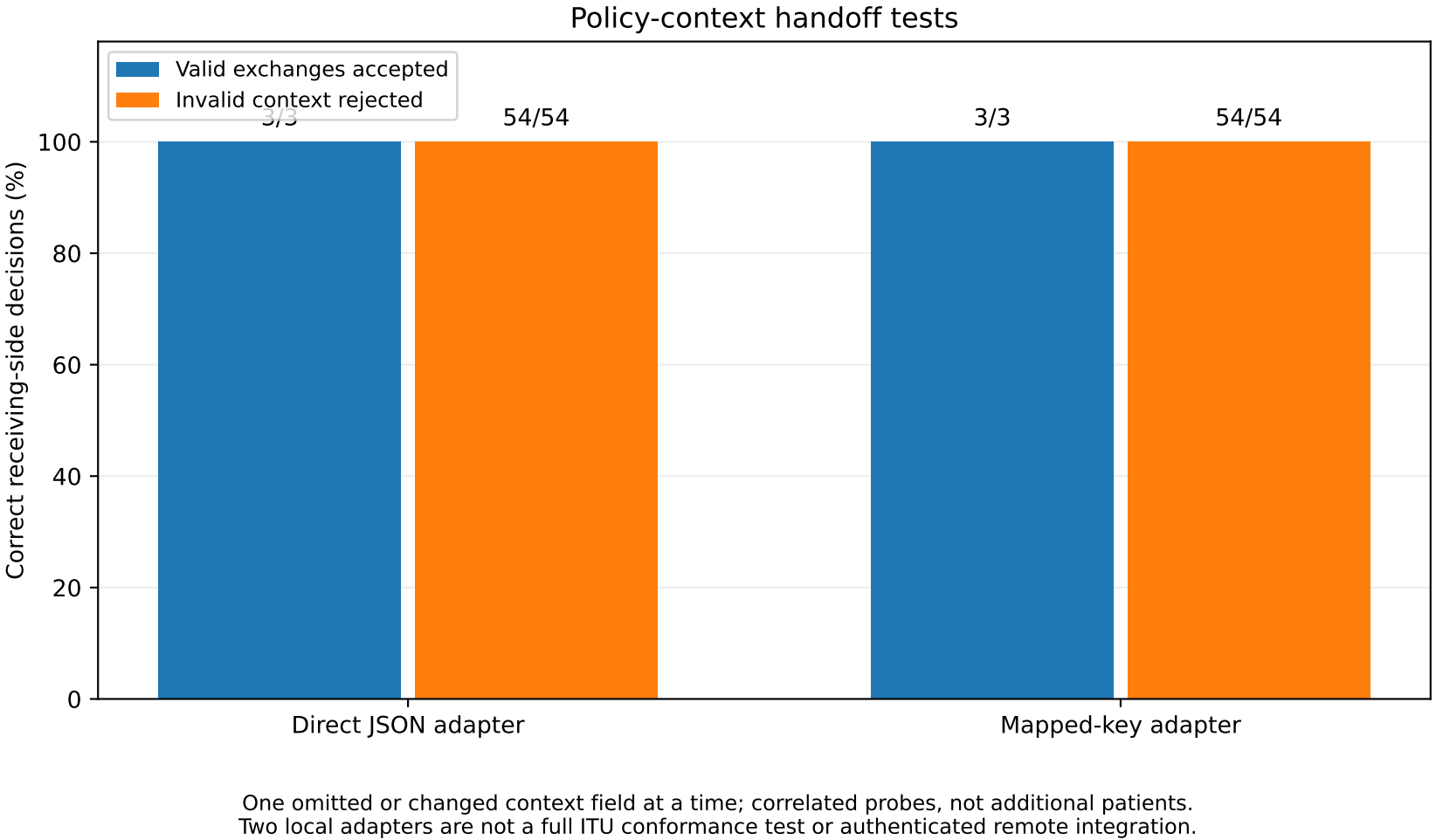
Engineering: PASS; 58/58 development checks. All new execution receipts were checked separately. Earlier project files preserved: 981/981. No classifiers were trained or loaded.

Important boundary

This is an additive deterministic research module, not a replacement for the live interface or its historical gates. Source snapshots are carried forward candidate records; substantive Korean applicability and clinical review remain pending. There is no new retrieval, signature-security, clinical-outcome, or full ITU conformance claim.


Gate 29: choosing an authorized workflow
Reference-feasible workflows completed (%)
0
20
40
60
80
100
1/10
10/10
10/10
10/10
Requested path only
Fair stateful selector
Independent constraint planner
MedVerse harmonizer
Fourteen internally authored confirmation families. Requested-path-only is a scope control.
Capable selectors retain ties. Reference outcomes do not establish clinical/legal validity.


Useful partial plans and uncertainty handling
Reference-optimal complete or partial plan
Correct evidence/authority disposition
Confirmation families
0
2
4
6
8
10
12
14
16
5
5
14
14
14
14
14
14
Requested path only
Fair stateful selector
Independent constraint planner
MedVerse harmonizer
Objective: preserve feasible clinical review, satisfy joint obligations, then minimize disclosure and steps.
Clinical and technical obligations can remain satisfied while research access is held.


Policy-context handoff tests
Valid exchanges accepted
Invalid context rejected
Correct receiving-side decisions (%)
0
20
40
60
80
100
3/3
54/54
3/3
54/54
Direct JSON adapter
Mapped-key adapter
One omitted or changed context field at a time; correlated probes, not additional patients.
Two local adapters are not a full ITU conformance test or authenticated remote integration.

Interpretation and next dependencies

Capable comparisons

Capable selectors agree with the reference objective and disposition: True. Capable selectors tie: True. A tie is retained, not converted to a superiority claim. Any benefit over the original-request control demonstrates alternative choice within this finite action library.

Temporal revalidation

42/42 change-before-commit probes rejected the affected final action. These are perturbations of the same families, not independent clinical cases. Modeled approvals cannot authorize an action after epoch, purpose, or policy binding changes.

Workspace handoffs

114/114 receiving-side decisions matched the separate implementation. Invalid or incomplete context accepted: 0. Explicit timestamps use an event-distance modeling assumption, not a clinical freshness threshold.

Source and institutional review

Interpretation review remains HOLD pending qualified Korean-policy and clinical reviewers. Clinical care-continuity obligations are explicitly modeled, not derived from a statute that sets a triage threshold. International WHO/OECD/ITU materials are comparison context, not Korean authorization.

Next dependency

Review and sign off the scenario contracts and reference outcomes; then separately integrate the tested selection module into the progressive UI with commit-time revalidation. Do not use these snapshot results as proof of a deployed end-to-end medical workflow. Any post-confirmation repair needs a new version and fresh confirmation.